

# Chapter 07: Living to Ninety

## Brief Summary

This chapter asks what predicted survival, health, and cognitive vitality into very old age. Vaillant distinguishes living long from living well, and he repeatedly challenges simple wellness slogans. The main lesson is that late-life physical health is shaped heavily by vascular and self-care factors established before age fifty. Love, mood, and social supports matter for happiness, but the strongest predictors of physical survival are more bodily and behavioral than Vaillant originally expected.

## Key Study Findings

- Of the seventy Grant Study men who reached ninety, fifty-eight were cognitively intact. Nearly three-quarters of the ninety-year-olds remained mentally sharp, though often slower.
- The main controllable predictors of healthy aging were vascular and self-care risks before fifty: smoking, alcohol abuse, obesity, hypertension, and diabetes.
- Men with no vascular risk factors lived to about eighty-six on average; men with three or more lived to about sixty-eight. This difference amounted to roughly eighteen years.
- Of men with no vascular risk factors, 50 percent were alive at ninety. Of the seventeen men with three or four risk factors, all but one was dead by ninety.
- Heavy smoking and alcohol abuse were especially lethal because they removed many men from the later-life sample before eighty.
- Education predicted longevity, partly because it was associated with better health behaviors and health literacy. Harvard men with postgraduate education lived longer than those without it.
- Ancestral longevity had a modest effect but did not dominate lifestyle. The longest-lived ancestor group lived only about seven years longer than the shortest-lived group.
- Cholesterol at fifty did not strongly distinguish those who lived to ninety from those who died before eighty.
- Warm childhood, social class, psychosomatic symptoms, and old-age social supports were less predictive of longevity than expected.
- Health often predicted exercise and social supports better than exercise or social supports predicted later health, complicating simple causal stories.

## Interesting Stories And Examples

- Alfred Paine is the negative case. He denied alcoholism and depression, smoked heavily, had poor intimacy, and reported himself healthier than objective evidence showed. His Decathlon score was zero, and he declined early.
- Daniel Garrick is the vivid positive case. Despite unconventional choices, late exercise, smoking history, and modest Decathlon scoring, he built a passionate late life through acting, teaching, marriage, sexuality, curiosity, and work.
- Garrick's eventual decline after retiring from theater shows the ambiguity Vaillant keeps returning to: did reduced activity cause decline, or did decline reduce activity? The long view still cannot solve every causal puzzle.

## Why It Matters

The chapter is one of the most practical in the book. For physical aging, the best advice is plain and early: stop smoking, treat alcoholism, control weight and blood pressure, prevent diabetes, and take education and health literacy seriously.